Patient Name	: **Mr. AVALSINH S NINAMA**	Type / IPNO	: IP / 60901
Gender / Age	: Male / 49 Years 7 Months 21 Days	Request No.	: **411750**
MR No / Bill No.	: **23290693 / 262129150**	Request Date	: 29/12/2025 09:42 AM
Consultant	: Dr. Nimesh Patel	Collection Date	: 29/12/2025 09:55 AM
	: Lavender Twin	Approval Date	: 29/12/2025 12:59 PM

Dengue NS1, IgM and IgG by ELFA

Test	Result	Units	Biological Ref.
Dengue NS1 ELFA			
Dengue NS1 ELFA	Negative(0.00)	Index	
Dengue IgM ELFA			
Dengue IgM ELFA	Negative(0.30)	Index	
Dengue IgG ELFA			
Dengue IgG ELFA	Positive(9.87)	Index	

Method: Enzyme Linked Fluorescent Assay (ELFA) on VIDAS
Guidelines for interpretation :
COI < 1 : Negative
COI => 1 : Positive

Detection of dengue NS1 (Non-structural protein-1) antigen allows detection of infection prior to seroconversion. NS1 antigen can be detected in serum from day 1 after onset of fever and up to day 9. It is also useful in possible secondary infection with high IgG and low or negative IgM.
Primary dengue infection is characterised by the presence of significant or rising levels of IgM 3-5 days after the onset of infection, which can persist for 3-5 months. Low levels of IgM antibodies may occasionally persist for more than 12 months post-infection. IgG antibodies are detected approximately 14 days after onset of primary infections.
Secondary dengue virus infection is characterised by high IgG levels detectable as early as 3 days, whose peak window of detection is 6-15 days following the onset of illness. This may be accompanied by elevated IgM levels. In early infections and some secondary infections detectable levels of IgM antibodies may be low.

The results of a single-specimen antibody determination should not be used to aid in the diagnosis of recent infection. Paired samples (acute and convalescent) should be collected and tested concurrently to look for seroconversion or a significant rise in antibody level, no later than 14 days after the first specimen. A negative result in immunosuppressed patients does not always exclude the possibility of infection. Samples showing equivocal results should be retested and samples that remain equivocal after repeat testing should be repeated by ELISA or RT-PCR method or another sample should be collected.

Assay results should be interpreted taking into consideration the patients clinical history and the results of any other tests performed.

Disclaimer :
It is agreed and implied with patient that certain information about positive lab result pertaining to infectious and communicable diseases may be shared with regulatory authority, as required.

---- End of Report ----

Dr. Rakesh Vaidya
MD (Path). DCP.